

1. Administrative & Study Identification

Full Study Title: An Exploratory Phase 1b/2a Multicenter, Open-Label, Novel-Novel Combination Study to Assess the Safety, Pharmacokinetics, Pharmacodynamics, and Preliminary Efficacy of CC-92480 (BMS-986348) in Novel Therapeutic Combinations in Participants with Relapsed or Refractory Multiple Myeloma **Short Title/Acronym:** CA057-003 **Protocol Number:** CA057-003 **NCT Number:** NCT05372354 **Sponsor Name:** Celgene Corporation / Bristol Myers Squibb **Investigational Product:** CC-92480 (Mezigdomide / BMS-986348) **Phase of Development:** Phase 1/Phase 2 (Phase 1b/2a) **Trial Status:** Recruiting **Medical Monitor:** [Name and Contact Information]

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To assess the safety, tolerability, and define the recommended phase 2 doses (RP2D) of CC-92480 (mezigdomide) combined with dexamethasone and novel targeted agents for the treatment of Relapsed or Refractory Multiple Myeloma (RRMM). **Secondary Objectives:** To evaluate preliminary efficacy including Overall Response Rate (ORR), Very Good Partial Response Rate (VGPRR), Complete Response Rate (CRR), Progression-Free Survival (PFS), Minimal Residual Disease (MRD) negativity, time-to-event outcomes, and Pharmacokinetics (PK).

2.2 Background & Rationale To address the unmet medical need in patients with highly refractory multiple myeloma who have exhausted standard therapies. Combining mezigdomide, an oral CELMoD™ agent, with other targeted agents (such as the EZH2 inhibitor tazemetostat, the BET inhibitor BMS-986158, or the MEK inhibitor trametinib) alongside dexamethasone intervenes on key dysregulated oncogenic pathways identified in RRMM to improve response rates and overcome drug resistance.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Single-arm (Dose finding and dose expansion cohorts) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration **Target N\$:** 260 **Number of Sites:** [Global and Regional breakdown] **Estimated Study Start:** [DD-MMM-YYYY] **Estimated Primary Completion:** [DD-MMM-YYYY]

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult men or women ≥ 18 years of age with Relapsed or Refractory Multiple Myeloma (RRMM) and documented disease progression during or after their last myeloma therapy. 2. For Part 1 (Dose Finding): Refractory to, intolerant to, or not a candidate for available established therapies known to provide clinical benefit in MM. For Part 2 (Dose Expansion): Refractory to or relapsed after prior lines of therapy including an immunomodulatory drug (IMiD), a proteasome inhibitor, an anti-CD38 mAb, and a T-cell redirecting therapy (TRT, eg, a CAR-T or T-cell engaging bispecific treatment) unless the participant is not a candidate for TRT. 3. Must have measurable disease and an Eastern Cooperative Oncology Group Performance Status (ECOG PS) of 0 or 1. 4. Agree to follow the CC-92480 Pregnancy Prevention Plan (PPP).

4.2 Exclusion Criteria 1. Known active or history of central nervous system (CNS) involvement of MM. 2. Plasma cell leukemia; Waldenstrom's macroglobulinemia; polyneuropathy, organomegaly, endocrinopathy, M-protein, and skin changes (POEMS) syndrome; or clinically significant light-chain amyloidosis. 3. Impaired cardiac function or clinically significant cardiac disease. 4. Previous SARS-CoV-2 infection within 14 days for asymptomatic or mild symptomatic infections or 28 days for severe/critical illness prior to Cycle 1 Day 1 (C1D1). 5. For Part 1: received prior therapy with CC-92480. 6. For Part 2: received prior therapy with CC-92480, tazemetostat, BMS-986158, or trametinib. 7. Previously received allogeneic stem-cell transplant at any time or received autologous stem-cell transplant within 12 weeks of initiating study treatment. 8. Received any of the following within 14 days prior to initiating study treatment: a. Plasmapheresis b. Major surgery c. Radiation therapy other than local therapy for myeloma associated bone lesions d. Use of any systemic anti-myeloma drug therapy 9. Used any investigational agents within 28 days or 5 half-lives (whichever is shorter) prior to initiating study treatment. 10. COVID-19 vaccine within 14 days prior to C1D1.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: CC-92480 (mezigdomide) specified dose on specified days, combined with Dexamethasone and either Tazemetostat, BMS-986158, or Trametinib. **Route of Administration:** Oral **Duration of Treatment:** Until disease progression or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard concomitant medications for co-morbidities. **Prohibited:** [Washout periods or strictly forbidden drugs]

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent, Medical History, Baseline Labs
Baseline	Day 0	First Dose, Safety Labs
Interim	Week [X]	PK/PD Assessments, Safety Monitoring (evaluating for neutropenia, anemia, infections), Efficacy Evaluation
End of Study	Week [X]	Final Vital Signs, Exit Interview, IP Accountability

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints * Number of participants with adverse events (AEs). * Number of participants with serious AEs. * Number of participants with AEs meeting protocol-defined dose-limiting toxicity (DLT) criteria to determine safety and maximum tolerated dose/RP2D.

Measurement Method: Medical monitoring, investigator assessment, and clinical laboratory analyses.

7.2 Secondary & Exploratory Endpoints * Overall Response Rate (ORR), Very Good Partial Response Rate (VGPR), and Complete Response Rate (CRR). * Progression-Free Survival (PFS) and Minimal Residual Disease (MRD) negativity.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via clinical evaluations, specifically monitoring for hematologic toxicities (e.g., neutropenia, anemia) and nonhematologic toxicities (e.g., infections, fatigue, dyspnea). **Serious Adverse Events (SAEs):** Reporting timeline, typically within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Internal/External committee details.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set, Intent-to-Treat (ITT) Set. **Sample Size Justification:** Powered to adequately assess dose-limiting toxicities and safety signals in Phase 1b, and preliminary efficacy/ORR in Phase 2a. **Handling of Missing Data:** [e.g., Last Observation Carried Forward (LOCF) or Multiple Imputation]

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: [Frequency of onsite vs. remote monitoring visits] **Audit Trail:** [Details on eCRF locking and data

clarification forms]

11. Study Identifiers & Governance

Primary ID: CA057-003 **Registry Number:** NCT05372354 **Sponsor/Lead Organization:** Celgene Corporation / Bristol Myers Squibb **Study**

Title: CA057-003: A Study to Evaluate Safety, Drug Levels and Effectiveness of CC-92480 (BMS-986348) in Combination With Other Treatments in Participants With Relapsed or Refractory Multiple

Myeloma **Official Regulatory Title:** An Exploratory Phase 1b/2a Multicenter, Open-Label, Novel-Novel Combination Study to Assess the Safety, Pharmacokinetics, Pharmacodynamics, and Preliminary Efficacy of CC-92480 (BMS-986348) in Novel Therapeutic

Combinations in Participants with Relapsed or Refractory Multiple Myeloma

12. Clinical Framework (Multiple Myeloma Specific)

Primary Condition: Relapsed or Refractory Multiple Myeloma (RRMM)

Diagnosis Threshold: Documented disease progression during or after the last myeloma therapy **Medical Methodology:** Targeted

CELMod™ Combination Therapy **Optimization Target:** Disease response optimization and toxicity management

13. Study Procedures & Methodology

Management Pathway: Clinical pathway for RRMM **Education Protocol (HCP):** Protocol training and AE (hematologic toxicity) management

Education Protocol (Patient): Informed consent and daily oral dosing instructions **Quality Audit Mechanism:** Clinical site

monitoring and safety dose-escalation reviews

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression or unacceptable toxicity **Onsite Visit Cadence:** Per protocol schedule (e.g., Day 1

of each treatment cycle) **Remote Monitoring Frequency:** As per

clinical monitoring plan **Checklist Review Interval:** Prior to each treatment cycle to assess Dose-Limiting Toxicities (DLTs)

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					